ToothReferral
Just Refer it

Draft

LINKEDIN CAROUSELS

3 posts for dental specialists.

Each post: the slides, then the caption, keywords and hashtags.

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FOR DENTAL SPECIALISTS

Most specialists think they need more GP referrals.

Your biggest leak is inside your own front office.

Swipe →

WHERE REFERRALS LEAK

One referral. Three ways to lose it.

A GP sends you a patient

Missed voicemail

Left on Monday. The caller is out sick Tuesday. The patient goes cold.

Wrong status click

Marked “Scheduled” instead of “Received”. It drops off every list.

Staff turnover

The coordinator quits. Their follow-ups leave with them.

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THE COST

Tens of thousands in lost production, every month.

Not from GPs who stopped referring. From patients they already sent you.

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TODAY: THE WEB FORM WORKFLOW

Nobody owns the referral.

Patient details sent through your web form

↓

An email lands in a shared inbox

↓

Someone has to notice it

↓

A call, a voicemail, then waiting

↓

✕ The case is forgotten

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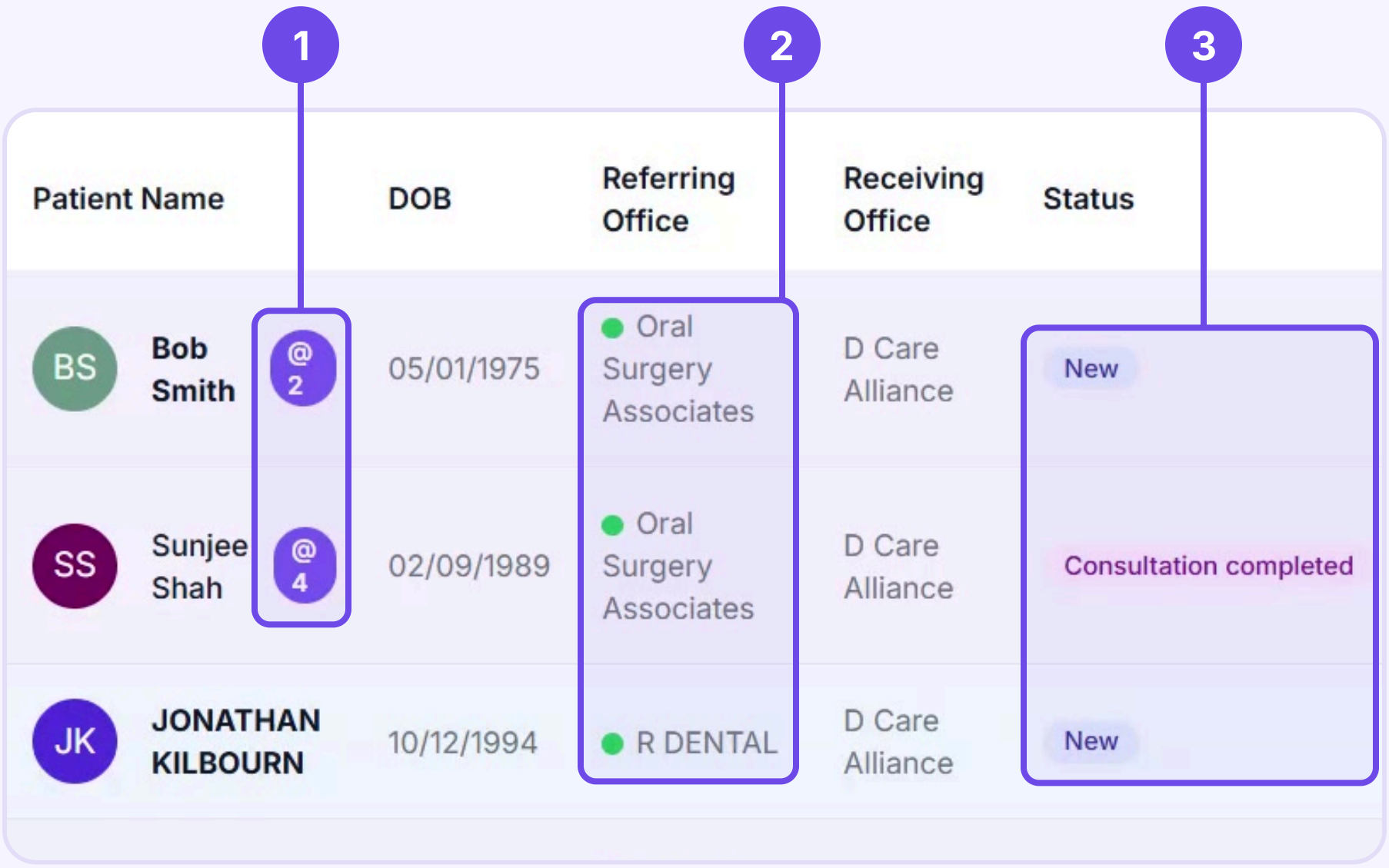
WITH TOOTHREFERRAL

Every step is tracked by the system.

- The referral lands in your shared dashboard
- Every call and voicemail is logged for the whole team
- “Scheduled” needs a real appointment and a note
- The referring GP is updated automatically

INSIDE THE DASHBOARD

Every referral, one list.



1. @ badge: your team was tagged in a message
2. Green dot: the referring office is on the network
3. The status of every case, at a glance

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THE RETURN

One saved referral pays for the system.

1 referral that doesn't leak

=

ToothReferral paid for

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EASY TO START

Live in about 5 minutes.

Your referral pipeline running for the whole team from day one.

Check out ToothReferral

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LINKEDIN CAPTION

Most specialists think they need more GP referrals. Their biggest leak is inside their own front office.

Look at what happens to the cases you already get.

A voicemail is left on Monday and the person who left it is out sick on Tuesday. Someone clicks "Scheduled" when they meant "Received." A coordinator quits and their follow-ups go with them. In a busy specialty office, that adds up to tens of thousands in lost production every month.

ToothReferral gives your whole team one referral pipeline. Every call, voicemail and status change is logged where everyone can see it, a case can't be marked Scheduled without a real appointment, and the referring GP is updated automatically.

One referral that doesn't leak pays for the system. Setup takes about five minutes.

#DentalMarketing #DentalPractice #DentalOfficeManager #Periodontist #OralSurgery

Keywords: dental referral software, specialist referral management, dental front office, lost production, staff turnover

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FOR SPECIALISTS

Referral portals fail.

They treat your referring doctors like outsiders who need an access code for every update.

Swipe →

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WHAT A GP GOES THROUGH FOR ONE UPDATE

Portals turn GPs into outsiders.

Click the portal link

↓

Enter the access code

↓

✕ “This code has expired”

↓

✕ Call your front desk anyway

THE TOOTHREFERRAL MODEL

Both offices on one platform.

GP office
own dashboard

ToothReferral

Your office
own dashboard

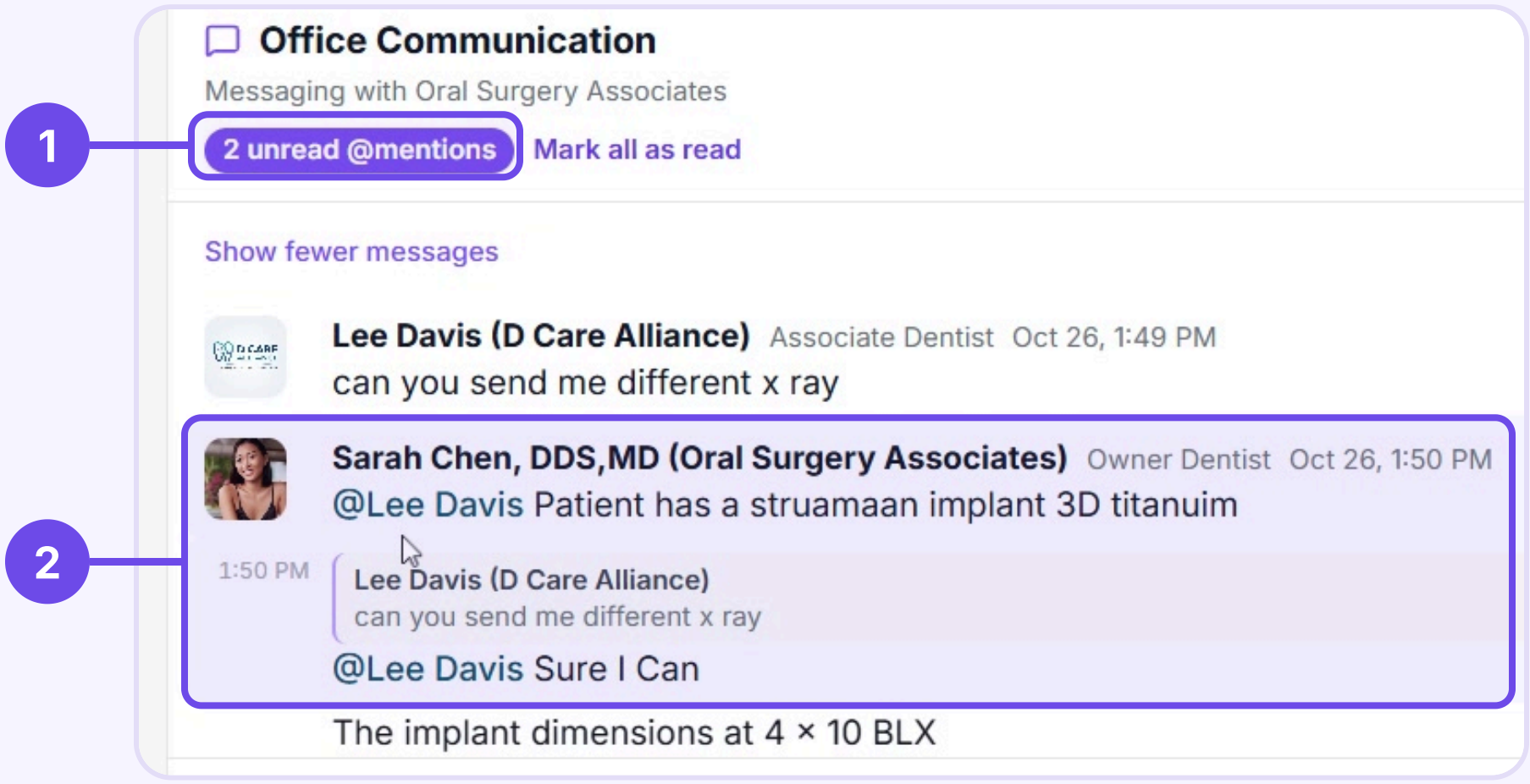
Shared instantly between both

Messages · X-rays · Status · Schedules

No access codes · No portal links · No redirects

INSIDE A CASE

Ask the GP. The answer stays with the patient.



1. **Unread @mentions for your team**
2. **The GP's reply, tagged to the right person, saved with the case**

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CLOSED LOOP

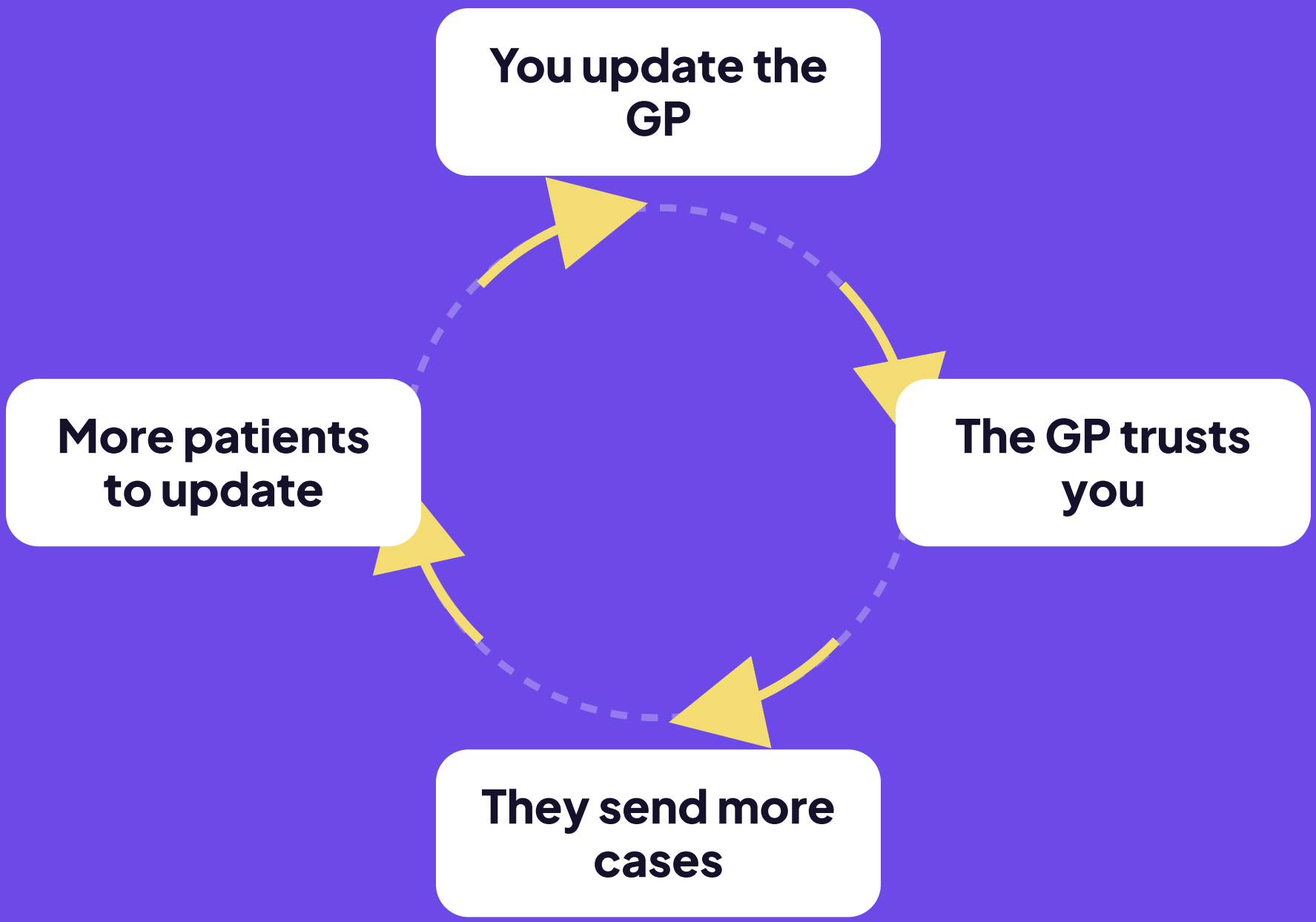
The GP sees every milestone in seconds.

- Case received — GP notified
- Outreach made — GP notified
- Consultation scheduled — GP notified
- Post-op complete — GP notified
- Returned for restorative care — GP notified

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WHY IT MATTERS

GPs don't leave over skill. They leave over silence.



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THE RETURN

One extra referral pays for the system.

Keep GPs informed, keep the referrals coming, and the software covers itself.

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TOOTHREFERRAL

Like Slack, built only for dental referrals.

Live in about 5 minutes. Invite your referring offices the same day.

Check out ToothReferral

LINKEDIN CAPTION

Referral portals fail because they treat your referring doctors like outsiders.

A GP who wants an update on a crown or an implant gets a portal link, an access code, and sometimes a code that expired last week. Many referring offices give up and call your front desk instead, which is the phone tag the portal was meant to end.

ToothReferral puts both offices on the same platform. The GP has their own dashboard and so do you. A case, a message or an X-ray shows up in both, with no codes and no links. Every milestone, from case received to the patient returning for restorative care, reaches the GP in seconds.

GPs rarely leave a specialist over clinical skill. They leave when they stop hearing back. Keep them updated and they keep sending cases, and one extra referral pays for the system. Setup takes about five minutes.

#GeneralDentist #Endodontist #DentalCommunity #DentalTech #DentalPractice

Keywords: dental referral portal, GP to specialist referrals, closed-loop referrals, referring dentist relationships, dental software

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FOR PRACTICE OWNERS

Is your referral pipeline built to transfer?

Or is it built on sticky notes?

Swipe →

WHAT BUYERS SEE: KEY-PERSON RISK

Your pipeline depends on one person.

Your referral pipeline today

One coordinator's memory
Who called, who's waiting, who needs a reminder.

Sticky notes and a private inbox
Nobody else can see what's pending.

They leave, the pipeline stalls
Buyers and DSOs price that risk in.

GUARDRAILS

No more accidental “Scheduled”.

Move this case to Scheduled?

Yes

✓

Appointment and note added: status moves, GP notified

No

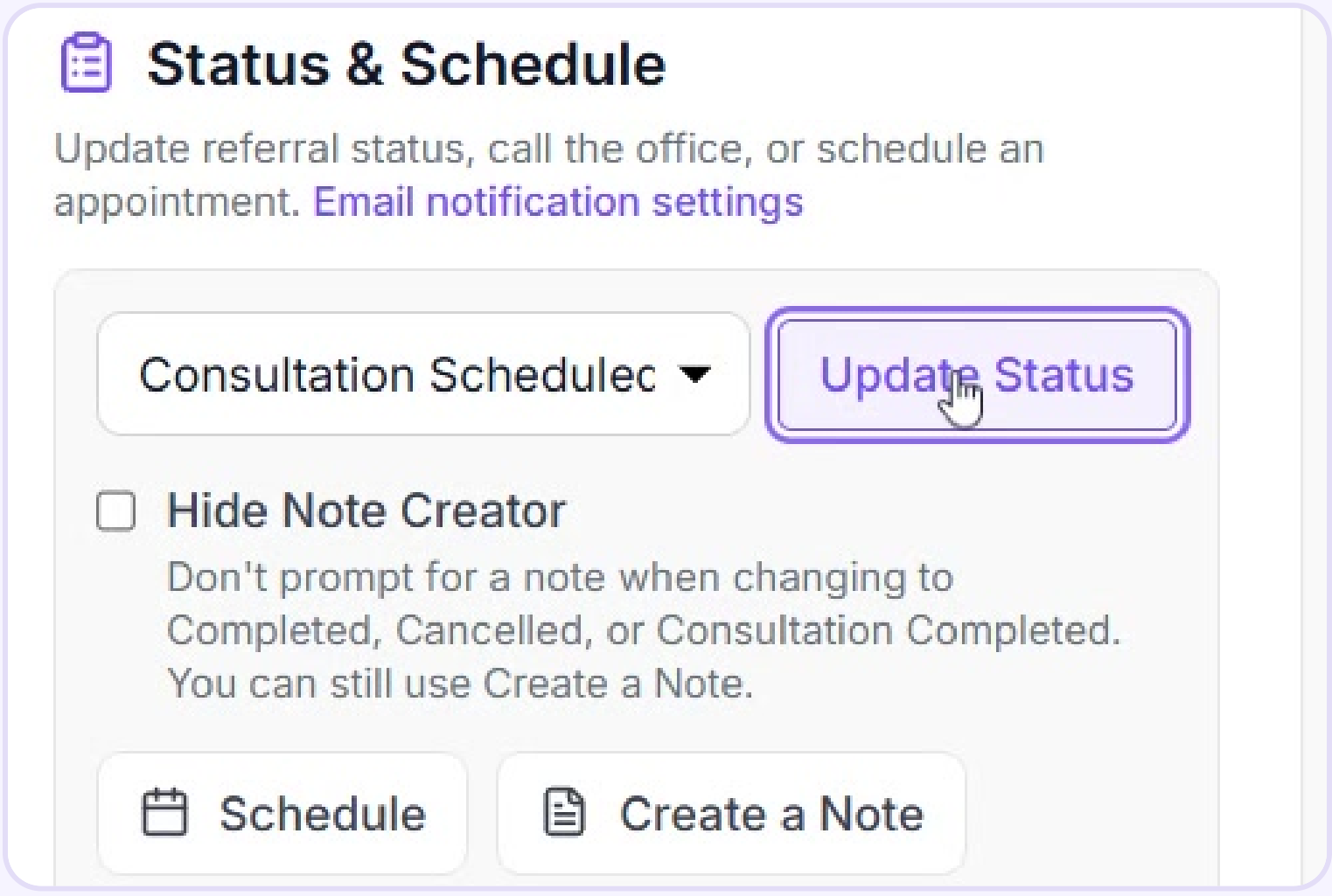
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Missing details: it stays in the team's queue

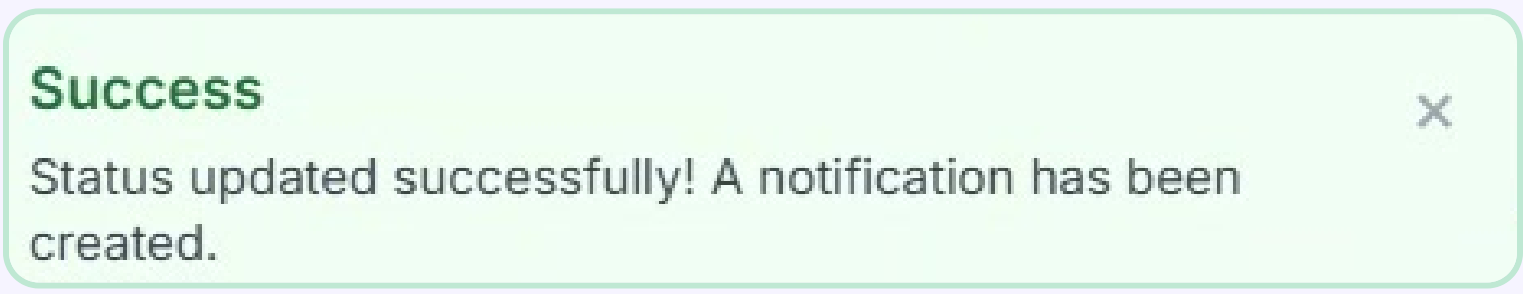
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INSIDE THE DASHBOARD

Update the status once. The GP is notified.



Then, instantly:



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DAY-ONE HANDOFFS

A new hire picks up where the last person stopped.

Coordinator logs a call and a voicemail

↓

The coordinator leaves the practice

↓

✓ New hire opens the dashboard

↓

✓ Sees the full history and follows up

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EVERY STEP IS LOGGED

Who did what, and when.

A new hire reads the history and carries on.

Activity log

- **Sarah Chen, DDS,MD** (10/26/2026 1:54 pm) patient insurance
- **Sarah Chen, DDS,MD** (10/26/2026 1:53 pm) patient insurance
- **Sarah Chen, DDS,MD** (10/26/2026 1:51 pm) updated: x-rays added
- **Sarah Chen, DDS,MD** (10/26/2026 1:50 pm) updated reason group: teeth, notes
- **Sarah Chen** (10/26/2026 1:48 pm) new referral created for D Care Alliance

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3 WAYS IT PAYS FOR ITSELF

Any one of these covers the cost.

ToothReferral pays for itself with

One new referral
covers the system.

One referral saved from leaking
covers the system.

One case saved when staff leave
covers it for a full year.

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AN ASSET, NOT A FORM

What you'd hand to a buyer.

Web form

- ✕ Tied to one person's habits
- ✕ No audit trail
- ✕ Stalls when staff leave

ToothReferral

- ✓ Transfers to a buyer
- ✓ Every step logged
- ✓ Anyone picks up any case

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EASY TO START

Live in 5 minutes.

Worth more every day after.

Check out ToothReferral

LINKEDIN CAPTION

Is your referral pipeline built to transfer, or built on sticky notes?

For many specialty practices, it's the sticky notes. The pipeline lives in one coordinator's head and an inbox only they check. Buyers and DSOs call this key-person risk, and they price it in.

ToothReferral keeps the pipeline in a system instead of a person. A case can't move to Scheduled without an appointment and a note, and every call, note and status change is logged, so a new hire can pick up where the last person stopped.

It pays for itself three ways: one new referral, one referral saved from leaking, or one case saved when someone leaves. Setup takes about five minutes.

#DentalPracticeManagement #DentalBusiness #PracticeOwner #DSO #DentalPractice

Keywords: dental practice valuation, selling a dental practice, key-person risk, dental practice management, dental referral ROI